Fortis
FLT. LT. RAJAN DHALL
HOSPITAL

Patient's N
UHID :
Age :
D.O.A. :

Mr. Rajeev Kakkar
63 Y/M; UHID: 10693597; IPID: 179296/25/1301
BED: ICU(2) 4; Domestic Insura; Dr.Neeraj Chaudhary/ Dr Nitin Kaushik/ Dr Harlom Sheoran

Unit :

MEDICATION ADMINISTRATION RECORD

Previous Hospitalization Yes ☐ No ☑	Weight in kg 100	Special Diet NPO
Blood Group :	Diagnosis:	
Drug Hypersensitivities/Allergies:	Surgery/ Procedure :	
	Date of Surgery:	

Standard timing: once a day: 10am, Twice a day: 10am - 10pm, Q8Hrly, 6am- 2pm - 10pm, Four times a day: 12am - 6am - 12pm - 6pm

Not to be use: U, IU, X.O, O.X QD, QOD, SL, CC, A/D, ATD, µg, MGSO4, MSO4

Note: Actual timing may vary as per ward routine. Administration of drugs half-an-hour before and after is acceptable

DRUG	Generic Name	Date	18/12		19/12							
INJ. MONOCEF		Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV											
Dose 2gm	Freq BD	10AM			10AM	PC RL						
Doctor's Name PRASANTHI	Signature / Time	10PM	10 PM	NJ								

DRUG	Generic Name	Date	18/12		19/12							
INJ. ESOMEPRAZOLE (SOMPRAZ)		Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV											
Dose 40mg	Freq BD	6AM			6AM	NJ SK						
Doctor's Name PRASANTHI	Signature / Time	6PM	6PM	NJ SK								

DRUG	Generic Name	Date	18/12		19/12							
INJ. ONDANSETRON		Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV	6AM			6AM	NJ SK						
Dose 8mg	Freq TDS	2PM										
Doctor's Name PRASANTHI	Signature / Time	10	8PM	SK								

DRUG	Generic Name	Date	18/12									
INJ. TRAMADOL		Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV	~~10AM~~				STOP						
Dose 50mg	Freq SOS											
Doctor's Name PRASANTHI	Signature / Time	~~10PM~~	10PM	NJ SK								

All Drug Names Will Be Written In Capital Letters By Doctor

FF LRDH/Nsg/20/IV-0919

DRUG	Generic Name		Date	19/12									
INJ. PEPCIN			Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV		10AM										
Dose 1AMP	Freq BD												
Doctor's Name PRASANTHI	Signature	Time	10PM										

DRUG	Generic Name		Date	18/12		19/12							
Inj. PARACIP			Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV		6AM			6AM	SK						
Dose 1gm	Freq TDS		2PM										
Doctor's Name MAYAMM	Signature	Time	10PM										

DRUG	Generic Name		Date	19/12									
Inj. OPTINEURON			Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV												
Dose 1 Amp	Freq OD		6AM	3AM	SK								
Doctor's Name MAYAMM	Signature	Time											

DRUG	Generic Name		Date	19/12/25									
Inj. NTC			Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions & 3 DAYS	Route IV		10AM	10AM	RL								
Dose 1gm	Freq BD												
Doctor's Name MAYAMM	Signature	Time	10PM										

DRUG	Generic Name		Date	19/12									
Inj. CALCIUM GLUONATE			Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV		6AM	3AM	SK								
Dose 1 Amp	Freq TDS		2PM										
Doctor's Name MAYAMM	Signature	Time	10PM										

DRUG	Generic Name		Date	19/12									
Inj. UTRIP			Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Special Instructions	Route IV		7AM	7AM	SK								
Dose 2 LAKH IU	Freq BD												
Doctor's Name MAYAMM	Signature	Time	7PM	7AM									

All Drug Names Will Be Written In Capital Letters By Doctor

PRIYA

As Required Prescriptions

DRUG	Generic Name		Date										
Special Instructions	Route		Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Dose	Freq												
Doctor's Name	Signature	Time											

DRUG	Generic Name		Date										
Special Instructions	Route		Time	Time	Initial	Time	Initial	Time	Initial	Time	Initial	Time	Initial
Dose	Freq												
Doctor's Name	Signature	Time											

Record of Drugs Not administered or Withheld

Date	Time	Name of Drug	Reason Not Administered	Ordered By Doctor	Nurse Initial

Stat / Once Only / Premedication Drugs

T. Flexyoe 4mg

Date	Drug (Approved Name)	Dose	Time	Route	Doctor's Signature	Given By	Checked by I / C
18/12	INJ. DEXON	1amp	9pm	IV		JASODA	
18/12	PARACETAMOL	1gm	8pm	IV	Dr. Manoj	JASODA	
18/12	INJ. NAC	2ml	8pm	IV	Dr. MANOJ	JASODA	
18/12	Inj. RINGER Infusion	@2ml/hr		IV		JASODA	
18/12	Inj. POTPHOS	1amp over 24 hrs	Infusion 1:30am	IV		JASODA	
18/12	Inj. LABET Infusion	@2ml/hr	8:30	IV		JASODA	
19/12	T. cibacar	1mg	8:30AM	ER		JASODA	
19/12	T. cibacar	1mg	8AM	ER		JASODA	

Date	Shift	Name of Nurse (in capitals)	Emp. Id.	Initials	Date	Shift	Name of Nurse (in capitals)	Emp. Id.	Initials
18/12	N	JASODA	7211	JJ					
19/12	M	PRIYANKA	5283	PC					
		(SORT)							

All Drug Names Will Be Written In Capital Letters By Doctor

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FLT. LT. RAJAN DHALL HOSPITAL

Emergency Department Nursing Assessment Sheet

Patient Name: MR. RAJEEV
UHID _____ IPID _____
Age 62 Sex Male
DOA 18/12/25

TRIAGE

General condition: conscious ✓ /unconscious ☐ /altered ☐ MOA: walking ☐ stretcher ☐ wheelchair ✓

CANADIAN TRIAGE & ACUITY SCALE

☐ **(Red) Resuscitation/Emergent:**
- Cardiac arrest ☐
- Respiratory arrest ☐
- ☐ Shortness of breath (Moderate respiratory distress)
- ☐ Hematemesis with shock/Tachycardia
- ☐ Hypertension (BP: SBP 220/DBP 130 with symptoms)
- Shock of any type (Cardiac/Non-cardiac)
- Major Trauma with shock ☐
- Shortness of breath (Severe respiratory distress ☐
- Altered level of consciousness (Unconscious GCS (3-9) ☐
- ☐ Fever with signs of Septic shock (qSOFA +ve)
- ☐ Chest pain with or without cardiac features
- ☐ Suspected stroke (Sudden onset within window period)
- ☐ Sudden severe headache/Abdominal pain (8/10)
- ☐ Major Trauma (Danger mechanism of injury)

(Yellow) Urgent:
- Shortness of breath (Mild respiratory distress)
- Vomiting/Nausea with dehydration
- ☐ Hypertension (BP: SBP 220/DBP 130 without symptoms)
- ☐ Abdominal pain Moderate (4-7/10)
- ☐ Headache (moderate 4-7/10)
- ☐ Uncontrolled/Bloody diarrhoea

(Green) Non-Urgent
- Confusion (Chronic no change from usual state)
- Diarrhoea with no dehydration
- Fever with stable condition
- ☐ UTI (Urinary Complain with fever/Back pain)
- ☐ Constipation with pain
- ☐ Cat 3 Dog bite
- ☐ Dressing change/Suture removal
- ☐ Medication request
- ☐ Mild Urticaria Skin rashes

BP: 120/80 mmHg RR 26 /Min HR 72 /Min Sp02 96 % RA/Fi02@ lit/min RBS 116 Mg/dl Temp: 97

Triage Category Red ☐ Yellow ☐ Green ☐ Black ☐

Handed over to Doctor Dr. Dileep Nursing Staff VISHNU Time 5pm

Date & Time of Arrival: 5pm 18/12/25

Mode of Arrival
☐ Wheelchair ☐ Ambulance ☐ Stretcher
☐ Self ☐ Brought by Police
☐ Attendant available ☐ Attendant not available

Date & Time of Assessment: 5pm 18/12/25

RAPID ASSESSMENT

Chief Complaints: Vomiting c̄ pain Abd.

Allergies: ☐ Yes ☐ No If yes, then Specify:

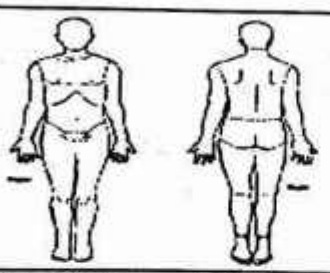
IMMUNITY
- Immune
- Visible
- Type: _____

SKIN ASSESSMENT
☐ Normal, warm, dry ☐ Cyanosed
☐ Clammy ☐ Pale ☐ Jaundice ☐ Cool

GCS Score (On Arrival)
E M V 15/15

PAIN SCALE
On Arrival: ☐ Yes ☐ No
If Yes, Pain Score: 1/10
Location: Abd
Time of Onset: Since morn

INTEGRITY
- (Location)
- Lesion
- Trauma
- Redness
- Trauma
- Bleeding
- F. Rash
- G. Laceration
- H. Hematoma
- I. Tender
- J. Other



LINES:
IV Cannula site: Rt
Gauge: 24G
Others:
Date of Insertion: 18/12/25

VISHNU

Pressure Injury present at the time of admission: ☐ Yes ☐ No
If Yes, Mention location, Size & stage: _____

Active Bleeding: ☐ Yes ☐ No If yes, then Specify:

Female Patients: LMP: NA GPLA: NA

INVESTIGATIONS

Blood: CBC KFT LFT Trop-I

Radiology: USG Sch NCCT W/A O

Others:

FHVK/ER/Nsg-Assessment/N.2/0925

VITAL SIGNS

Time	Temperature	Heart Rate	Respiratory Rate	BP	SPO2	Pain Score	GCS	RBS	Pupil RT	Pupil LT
5pm	97	74	24	132/70	91	1/10	15/15	113	R	L
6pm	96	76	26	132/70	98	0/10	15/15	—	R	R

	PARENTERAL INTAKE			INTAKE		OUT PUT				
TIME	IV FLUIDS	RATE	AMOUNT INFUSED	TIME	ORAL	TIME	URINE	EMESIS	NG	OTHERS

DISCHARGE/TRANSFER DETAILS ☐ DISCHARGE ☑ TRANSFER ☐ LAMA ☐ DEATH ☐ REFERENCE

Vitals:
Temperature: 97 HR: 74 RR: 24 BP: 132/70 SPO2: 96 GCS: 15/15 RBS(Relevant): 106

Nurses Progress Notes:

→ PT CAME IN ER C/O PAIN ABD AND VOMIT
→ INFORM TO DR
→ ALL MEDICATION GIVEN AS PER ADVICE
→

Additional Discharge/LAMA/Transfer Notes:

→ EXPLAIN PROCEDURE TO PATIENT
→ SIDE RAIL UP
→ PT SHIFTED TO ICU & HANDOVER GIVEN TO STAFF BY - VISHNU AT 1:30 pm 18/12/25

Completed by Nurse Name: VISHNU EMP ID: 1020203 Signature:

Name of Unit in charge: Emp ID: Signature:

FHVK/ER/Nsg-Assessment/N.2/0925

RLE/LLE

MW = Mild Weakness
SW = Severe Weakness
T = Track

Clear = C
Crepts = E
Wheeze = W
Decreased = D

EXTREMITIES
Warm = W
Cool = C
Diaphoretic = D
Clammy = CL
Cold

Site: W CD C CL D

Site: W CD C CL D

Site: W CD C CL D

Inserted on:
IABP
Inserted on:
Pacemaker

or toes cool, pale or cyanotic, numb, tingling reported,
movement, color returns slowly
or toes cool, pale or cyanotic, color returns with
g. normal, movement and sensation,
or toes cool and pale, which return of

Red
Yellow
Green
Black

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FLT. LT. RAJAN DHALL
HOSPITAL

TRIAGE SHEET

Patient's Name : Mr Rajeev

UHID 53 Y/M; UHID: 10693597; IPID: 179296/25/1301
ED: TRIAGE 11; Domestic Insura; Dr.Neeraj
Age : Chaudhary/ Dr Nitin Kaushik/ Dr Hariom Sheoran
Diagn Mr. Rajeev Kakkar

Date of Examination : 18/12/25

Time of Examination :

History Given by Patient/Attendant/Passer By/Police

TRIAGE :

☐ EMERGENT (Critical) ☐ URGENT (Unstable Guarded) ☐ NON - URGENT (Stable) ☐ DECEASED

DATE / TIME OF TRIAGE COMPLICATION

MLC No. : Non MLC

P.S. :

Date of Occurrence :

Time of Occurrence :

Police Notified : ☐ Yes ☐ No

Mode of arrival :
☐ Ambulatory ☐ WC ☐ Stretcher
☐ Other (Specify)

Conditions on Arrival : ☐ Alert ☐ Vebal Stimuli ☐ Painful Stimuli ☐ Unconscious ☐ Other (Specify)

Chief Complaints :

c/o pain abdomen c
Nausea &
Vomiting 2-3 episod
watery GD
generalized weakness
for 2 days

No c/o fever/cough/SOB/Loose motion

Current Medication :

Allergies : —

Last Meal :

Personal hz : ☐ Smoking : ☐ EtOH : ☐ Others :

Obstetric hx (where appropriate) : LMP G P A D

Past med hx : ☐ DM ☐ HTN ☐ CAD ☐ COPD ☐ BA ☐ CRF ☐ CLD ☐ CVA ☐ Other (Specify) :

BP	HR :	RR :	SpO2 :	Temp(°F) :	Pain Score :	RBS :
110/74 Supine Erect	62/min	20	98%	☐ Ax ☐ Oral ☐ PR 97.4	4/10	151 mg/dl

FFLRDH/ER/04-0222

General Examination :

Pallor _____ Cyanosis _____ Icterus _____ Ln'pathy _____ Padal Edema _____ JVP _____

Other (Specify)

Mental Status : Conscious/oriented

Oriented to Time Place Person Event

HEENT [illegible]

CVS : S1S2

RS : B/L ⊕

P/A Soft, [illegible]

Genitallia/Pelvic [illegible]

Extramities (Incl. Pulses) :

CNS. Pupils, Size : R (P) **L** (P)

Reaction (Brisk/Sluggish/Unresponsive) : R BL **L** BL

GCS E 4 **V** 5 **M** 6

Deficits : _____

Other Recevant Findins : _____

Eye Opening	Spontaneous	4
	To Voice	3
	To Pain	2
	None	1
Verbal Response	Oriented	5
	Confused	4
	Inappropriate Words	3
	Incomprehensible Sound	2
	None	1
Motor Response	Obeys Commands	6
	Localized Pain	5
	Withdraw to Pain	4
	Flexion to Pain	3
	Extension to Pain	2
	None	1

= Fingers or toes cool, pale or cyanotic, numb, tingling reported, slow movement, color returns slowly
= Fingers or toes cool, pale or cyanotic, color returns with movement and sensation.

Diaphoretic = D
Clammy = CL
Cold

ECG :

INVESTIGATIONS

ABG : (Time)	SO2	pH	pCO2	pO2	HCO3	SBE	cHb

Speciality Response :

Time of Call :	Doctor's Name :	Speciality :	Time of ER Visit :

Admit to Dr Neeraj Choudhary (HDU)

Investigations :	Problem List / Provisional Diagnosis / Treatment Plan :
USG W/A	Δ Acute pain abdomen
CECT W/A	

Checklist at Transfer

- ☐ IV Access
- ☐ ETT
- ☐ Foley's
- ☐ Cardiac Monitor/Defib
- ☐ Airway
- ☐ Shifting Vitals
- ☐ Oxygen
- ☐ NG/OG
- ☐ Others

Shifting Vitals HR BP SpO2 RR GCS

Transfer to : ☐ Discharge ☐ LAMA Ward ICU Mortuary

Admitting Consultant : Dr Neeraj Choudhary (HDU)

Dr Dilee

ER Physician's Name / Signature / ID

FFLRDH/ER/04/3-0222

ER MEDICATION ADMINISTRATION RECORD

Sl. No.	Date & Time	Drug Name (Capital Letter)	Dose	Frequency	Route	Doctor Name & Signature	Administered by (Name & ID)
	18/12/24	IV PANTOP 40mg		Daily			VISH 9:00 pm
		IV [illegible]		[illegible]			VISH [illegible] "
		IV [illegible] 1G		[illegible]			VISH [illegible] "
		IV Pesirona 1G		[illegible]			VISH [illegible] "

FFLRDH/ER/04/3-0222

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HOSPITAL

In Patient History and Physical Record

Patient's Name :
BED: TRIAGE 11; Domestic Insura; Dr.Neeraj Chaudharv/ Dr Nitin Kaushik/ Dr Harlom Sheoran
UHID :
Age :
Mr. Rajeev Kakkar
63 Y/M; UHID: 10693597; IPID: 179296/25/1301
BED: TRIAGE 11; Domestic Insura; Dr.Neeraj
D.O.A. :

Direct Admission / Shifted From Triage

Date of Admission : Time of Admission : Occupation :

Consultant : Speciality :

ant : Relationship : Father/Mother/Other

lity : Good ☐ Fair ☐ Poor ☐

s (if Any) : cough c̄ sputum - 2days.

omplaints : Pain Abdomen → Since morning
Vomiting 1-2 Episodes
Generalised weakness → 2-3 days.

History of Present Illness : (If pain is present, please describe location, rating, character etc.)

63yrs / Male was apparently well 3 days ago. Started feeling generalised weakness. Had Abdominal pain with Nausea & vomiting since morning. Non bilious vomiting containing food particle. H/o cough c̄ productive sputum for 2days has come to EC of FHVK & was shifted to HOU for further management.

dical / Surgical History : (Year/Procedure/Hospital/Anaesthesia)

Personal History :

Menstrual History : (If applicable)

Age at menarche : **Menopause :** **Cycler : Regular/Irregular Any abnormalities :**

Nurtritional assessment :Weight: **Kg.** **Height:** **cms.**

Weight appropriate for gender, height and age : Yes ☐ **No** ☐

+3 = Strong
+4 = Bounding
RLE/LLE
MW = Mild Weakness
SW = Severe Weakness
T = Track

Restraints

CSM: 0 = Fingers or toes - slowed mo...
CSM: 1 = Fin...
... numb, tingling reported.
CSM ...
...with

Clear	=			
Crepts	=	C	W D	C E W D
Wheeze	=	E		
Decreased	=	W		
		D		
EXTREMITIES			Site	Site
Warm	=	W	W CD	W CD
Cool	=	C	C CL	C CL
Diaphoretic	=	D	D	D
Clammy	=	CL		
Cold	=			

Inserted on: ___
IABP
Inserted on: ___
Pacemaker

Doctor Signature

...atory : Veg ☑ Non Veg. ☐

...d therapeutic diet: Therapeutic Diet (Specify) ☐

...d Family History :

...ficant ☑ Congenital Anomalies ☐

...ension ☐ Diabetes ☐

...hsocial History : Malignancy ☐ Tuberculosis ☐

...ence Abuse. Mental Illness ☐ Any Other : ☐

...ko : Yes ☐ No ☑ (If Yes, Specify)_____

...d Yes ☐ No ☑ (If Yes, Specify)_____

Yes ☐ No ☑ (If Yes, Specify)_____

Yes ☐ No ☑ (If Yes, Specify)_____

...onal assesment : Weight Kg : Height : Cms :

...t appropriate for gender, height and age : Yes ☐ No ☐

...Consciousness : Conscious ☑ Drowsy ☐ Unresponsive ☐ GCS ☐ EMV ☐

...Signs.

Temp-F	Pluse/HR-min	RR-min	BP - mm Hg	Pain Score 0-10 scale	Spo2
Afebrile	69/min	20/min	110/70 mmHg		98%

...ral Description

...Pluse / Extremities : WNL ☑

...d and Neck : WNL ☑

...st/Breast : WNL ☑

...men : WNL: ☐ P/A - soft, mild tenderness (+)

...k (Including spine) : WNL ☑

...t : WNL ☑

...rological Status : WNL ☑

...t : WNL ☑

...tal / Rectal : WNL ☑

...al Status : WNL ☑ Oriented to time, place and person : Yes ☑ No ☐

PROVISIONAL DIAGNOSIS (S) ? SAP ? Acute Pancreatitis
? Renal Calculi

INVESTIGATIONS
BeOP Extended Panel, LFT, Ca^{2+}, Mg^{2+}, IO_4^-, CXR, CRP.
USG W/A, CECT W/A, Hm 2g r to Blood c/s, S. amylase & lipase
Xray Abd

TREATMENT PLAN
Fluids Infusion @2ml/hr (500cc/500ml Dlcn)
Inj NAC 2gm iv stat & 1gm iv BD

DIET PLAN

DISCHARGE PLANNING
(To be filled by doctor completing History & Physical)

Lives with : Spouse / Adults Children / Minor Children / Others

Support systems at home : Present / Absent (Specify) Care Giver

will patient require transportation arrangement to go home : Yes /No

Stair to climb at home : Yes / No No of floor :

will Physiotherapy be required at home? Yes / No

Is home medical equipment anticipated? Yes / No

Is home Oxygen therapy anticipated? Yes / No

Are dressing needs at home anticipated? Yes / No

Any other needs anticipated :

Social worker consultation need Date and time requested

Doctor's Signature : Name : Date and Time

Consultant's signature : Name : Date and Time

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FLT. LT. RAJAN DHALL
HOSPITAL
DOCTOR'S PROGRESS / ORDERS SHEET

Patient's Name : Mr Rajeev
UHID : Mr. Rajeev Kakkar 63 Y/M; UHID: 10693597; IPID: 179296/25/1301 BED: ICU(2) 4; Domestic Insura; Dr.Neeraj Chaudhary/ Dr Nitin Kaushik/ Dr Harlom Sheoran
Age :
D/O/A : Mr. Rajeev Kakkar Unit : Pg :

DATE/ TIME	
18/11/20 10 pm	UHB GI Surgery team
	c/o
	Severe abdomen pain x 6 hrs
	- severe abdomen pain in epigastric & Lt. lumbar region alongwith nausea & not passing of flatus & stool x 1 day.
	- No fever, Bleeding PR, diarrhoea, ch. distension
20/11/20 10 am	- No any past medical / surgical history
	-
37y/M	O/E - Conscious, oriented. tongue dry
	PA - tenderness (+) in epigastric
	Ad
	- NPO
	- IV/F NS 100 ml/hr
	- Inj Monocef 1gm iv BD
P.T.O	- Inj pantop 40mg iv BD

FFLRDH/Med/06/11-0207

FFLRDH/Med/06/11-0207

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DOCTOR'S PROGRESS / ORDERS SHEET

Patient's Name :
UHID : IPID :
Age : Sex :
D/O/A : Unit : Pg :

DATE/ TIME	
	Inj. Emeset 8 mg in IV
	Inj. Protin Lans in 50ml → BD
	Inj. Tramadol 50 mg in 50 BD / Alternate
	CBC, LFT, RFT, TMK, HbA1c, HIGA, HCV, HbsAg
	USG abdomen
	* = CECT Abdomen with IV contrast + oral contrast (Iohexol) (+ve contrast)
	= Admit to Dr. Neeraj Chaudhary ↓ desired category
	ICU

Dr. Neeraj

FFLRDH/Med/06/11-0207

mal, movement and sensation.

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Patient's Name : Mr. Rajeev Kakkar
63 Y/M; UHID: 10693597; IPID: 179296/25/1301
BED: ICU(2) 4; Domestic Insura; Dr.Neeraj Chaudhary/ Dr Nitin Kaushik/ Dr Hariom Sheoran

Mr. Rajeev Kakkar
63 Y/M: UHID: 10693597; IPID: 179296/25/1301

D/O/A : _____ Unit : _____ Pg : _____

DATE/ TIME	
18/12/2025 9:13 pm	~~CFS (B) LI Sx fever~~
	→ Rylin tube need to put.
	→ vomiting
	→ plan for CECT w/A I/V + oral contrast
	Ash
	→ Oral contrast by oral.
	→ put Foley's catheter & urosafe melon.
	→
	Dr
	MB Ganguly (725) 18/12/25, 9:13 pm

FFLRDH/Med/06/11-0207

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DOCTOR'S PROGRESS / ORDERS SHEET

Patient's Name :
UHID : IPID :
Age : Sex :
DOA : Unit : Pg :

DATE/TIME

UHR OT Surgery team

- Acute pancreatitis

pt is AF-5

Abd - tenderness ⊕ in epigastrium

Symptomatically better

BP - 140/85
P - 105
SpO2 - 94%

Hb - 23.8
Urea - 85 mg/dl

RT - 150 ml

Advice - Lipid profile to be done on next prick.
- CBS
- Keep NPO
- DVT Pump.
- Collect CECT report
- Incentive spirometry
- chest ⅓ Cinch physiotherapy

Dulcolax suppository
⊕ 8 feet

NOTED BY
PRIYANKA @ 10 AM

FFLRDH/Med/06/11-0207

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HOSPITAL
DOCTOR'S PROGRESS / ORDERS SHEET

Patient: [illegible] Chaudhary/ Dr Nitin Kaushik/ Dr Harom Sharma
UHID: Mr. Rajeev Kakkar
33 Y/M; UHID: 10699597; PID: 279296/25/2301
Age: RFD- TRIAGE 11: Domestic Insura: Dr.Neeraj
D/O/A: _____ Unit: _____ Pg: _____

DATE/ TIME	
19/12/25 12.10pm	c/s/B Dr Shubham
	PT-reviewed
LFT (N)	Acute Pancreatitis
U/O 2850ml/24hr	Not passing flatus.
	Symptomatically better
	Vitals stable
	Keep NPO
	CS 7
	[illegible]

FFLRDH/Med/06/11-0207

Fortis
FLT. LT. RAJAN DHALL
HOSPITAL
DOCTOR'S PROGRESS / ORDERS SHEET

Patient's Name : _____
UHID : _____ IPID : _____
Age : _____ Sex : _____
D/O/A : _____ Unit : _____ Pg : _____

DATE/ TIME	
19/12/25 12 noon	Exit Note Patient NPO till further order. [signature]

FR.ROHMed/05/11-2007

MAHAJAN IMAGING

FORTIS
Flt. Lt. Rajan Dhall Hospital
B1, Vasant Kunj, New Delhi-110070
☎: 011- 42776222 Ext. 5813, 5815
info@mahajanimaging.com
www.mahajanimaging.com
CIN : U85199DL1999PTC101010

Patient Name	:	Rajeev Kakkar	Scan Date	:	19-12-2025
Sex	:	M	Patient ID	:	10693597
Age	:	63Y 10M 13D	Modality	:	CT
Accession No.	:	IP.46050133	Referring Physician	:	Dr. Neeraj Chaudhary / Dr. Nitin Kaushik / Dr. Hariom Sheoran .

CECT WHOLE ABDOMEN

The examination was conducted on a 128 slice multidetector CT (MDCT) scanner. Serial axial sections were obtained from the domes of diaphragm till the pubic symphysis after IV administration of non-ionic contrast (70ml of Omnipaque 300mg/ml). KFT – Serum creatinine 1.1mg/dL. Oral contrast was also administered for stomach opacification. No contrast reaction was observed.

Clinical profile: Pain abdomen and vomiting.

FINDINGS:

The liver is normal in size, contour and contrast enhancement. **It shows diffuse fatty attenuation.** No obvious focal hepatic parenchymal lesion is seen. Portal vein is normal in caliber. No IHBR dilatation. CBD is normal in calibre.

Gall bladder is unremarkable. There is no obvious radio opaque calculus in gall bladder (the radiolucent calculi however may not be detected on CT).

Pancreas is mildly bulky with peripancreatic fat stranding and minimal fluid collection along the lesser sac – acute pancreatitis. Advised amylase / lipase correlation.

Spleen is normal in size and attenuation.

Bilateral adrenals appear normal.

Bilateral kidneys are normal in position, size and attenuation. No evidence of hydronephrosis / ureteric dilatation on either side. No evidence of calculus.

Urinary bladder is empty with Foley's bulb.

Prostate is enlarged. Bilateral seminal vesicles are normal.

Oral positive contrast is seen reaching till distal small bowel loops. Visualized bowel loops are normal. No evidence of abnormal dilatation/narrowing.

No evidence of significant abdominal lymphadenopathy.

No evidence of free fluid in abdomen.

Please correlate clinically.

DR. DEVENDRA K. SINGH, DMRD, DNB
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(CONSULTANT RADIOLOGIST)
DMC NO. 112836

DR. ANUBHAV, MD
(CONSULTANT RADIOLOGIST)
DMC NO. 78903

Please correlate the measurements on the typed report with the images and in case of any discrepancy / doubt, please contact us immediately. There is o
professional opinion and should be correlated clinically. Not valid for medico-legal purpose.

Fortis Hospital, Vasant Kunj • Dwarka • Safdarjung Development Area • Defence Colony • Pusa Road • Gurugram
• PSRI Hospital • Fortis Lafemme Hospital, GK-2 • Fortis Escorts Hospital, Jaipur • ILSS Hospital, Faridabad

• 1.5 Tesla Advanced MRI • Functional MRI • Cardiac MRI • 128 Slice MDCT • 5 Second Heart Scan • True Digital X-Ray, IITV • 3D/4D Ul
• Digital Mammography • Colour Doppler • DEXA Bone Densitometry • Nuclear Medicine Scans • Dual Head SPECT Gamma Cam

C. VASCULAR Heart Sound			Night Time
	Normal / Abnormal	Normal / Abnormal	Normal

PATIENT NAME : MR.RAJEEV KAKKAR

FLT LT RAJAN DHAL CHARITABLE TRUST IPD
SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ,
NEW DELHI 110070
11-42776222

ACCESSION NO : 0013YL005858
REF. DOCTOR :
PATIENT ID : FH.10693597
CLIENT PATIENT ID: UID:10693597
ABHA NO :

AGE/SEX : 63 Years Male
DRAWN : 18/12/2025 20:24:00
RECEIVED : 18/12/2025 20:52:35
REPORTED : 18/12/2025 21:56:33

CLINICAL INFORMATION :

UID:10693597 REQNO-24041022
IPD-ICU(2)
IPID-179296/25/1301
IPID-179296/25/1301

Test Report Status **Final**

Results

		Biological Reference Interval	Units
BIOCHEMISTRY			
CALCIUM, SERUM			
CALCIUM	4.7 Low	8.8 - 10.2	mg/dL
MAGNESIUM, SERUM			
MAGNESIUM, SERUM	2.6	1.6 - 2.6	mg/dL
PHOSPHORUS, SERUM			
PHOSPHORUS	1.4 Low	2.7 - 4.5	mg/dL
LIPASE, SERUM			
LIPASE	1155 High	13 - 60	U/L
AMYLASE,SERUM			
AMYLASE	374 High	28 - 100	U/L
CRP (C-REACTIVE PROTEIN),SERUM			
C-REACTIVE PROTEIN	3.4	< 5	mg/L

Interpretation(s)
CALCIUM, SERUM-**Common causes of decreased value of calcium (hypocalcemia)** are chronic renal failure, hypomagnesemia and hypoalbuminemia. **Hypercalcemia (increased value of calcium)** can be caused by increased intestinal absorption (vitamin D intoxication), increased skeletal reabsorption (immobilization), or a combination of mechanisms (primary hyperparathyroidism). Primary hyperparathyroidism and malignancy accounts for 90-95% of all cases of hypercalcemia. Values of total calcium is affected by serum proteins, particularly albumin thus, latter's value should be taken into account when interpreting serum calcium levels. The following regression equation may be helpful.
Corrected total calcium (mg/dl)= total calcium (mg/dl) + 0.8 (4- albumin [g/dl])*
The possible mathematical corrections should be replaced by direct determination of free calcium by ISE. A common and important source of preanalytical error in the measurement of calcium is prolonged torniquet application during sampling. Thus, this along with fist clenching should be avoided before phlebotomy.
MAGNESIUM, SERUM-Description- Magnesium is primarilly an intracellular ion associated with GI absorption and renal excretion. Second most abundant ion in bone.It functions as co-factor in numerous enzymes e.g. ATPase . 65-70% of Mg is in ionized state and nearly 35% is protein bound.

Interpretation-
Increased in- Dehydration, Tissue trauma, Renal failure, Hypothyroidism, excessive intake of antacid.
Decrease in- Chronic diarrhea, Enteric fistula, Starvation, Chronic alcoholism, Total parenteral Nutrition, Diuretics.

Note- Hypomagnesemia is associated with weakness, tetany, disorientation and somnolence

Limitation-
* Hemolysis yields elevated levels of Mg being an intracellular ion.
* Serum magnesium levels may remain normal even when total body stores of magnesium are depleted upto 20%
LIPASE, SERUM-Lipase is a glycoprotein enzyme. This test is useful for investigations of pancreatic disorders, usually pancreatitis however serum level of lipase is increased in both pancreatic and non-pancreatic causes like acute pancreatitis, pancreatic duct obstruction, chronic pancreatitis, perforated or penetrating peptic ulcer, acute cholecystitis, small bowel obstruction,intestinal infarction as well as in renal failure.

Values remain normal in parotitis and in macroamylasemia.

Dr Nitin Kumar Dumeer, M.D.
FRCPATH Histopathology (Reg No. DMC 30700)
Lab Head

Dr.Ruchika Majoka
Consultant Pathologist

View Details View Report

ULR No.13000002450674-0013

MC-5836

PATIENT NAME : MR.RAJEEV KAKKAR	REF. DOCTOR :	AGE/SEX :63 Years Male
FLT LT RAJAN DHAL CHARITABLE TRUST IPD SECTOR-B, POCKET-1, ARUNA ASAF ALI MARGVASANT KUNJ, NEW DELHI 110070 11-42776222	ACCESSION NO : 0013YL005858 PATIENT ID : FH.10693597 CLIENT PATIENT ID: UID:10693597 ABHA NO :	DRAWN :18/12/2025 20:24:00 RECEIVED : 18/12/2025 20:52:35 REPORTED :18/12/2025 21:56:33

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UID:10693597 REQNO-24041022
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IPD-179296/25/1301
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Test Report Status **Final**	Results	Biological Reference Interval	Units

In acute pancreatitis serum lipase starts to increase within 3-6 hrs, reaches peak at 24 hrs and remains elevated for 8-14 days.Critical elevation is usually considered when the levels are three times the upper limit of normal. Since amylase levels are apt to return to normal range first, serum lipase estimation is especially helpful in patients presenting late with symptoms.

AMYLASE,SERUM-Amylase enzyme is of predominantly pancreatic or salivary gland origin. Amylase levels increase in acute pancreatitis, pseudo- cyst of pancreas, obstruction of pancreatic ducts, mumps, occasionally elevated in renal insufficiency, ruptured ectopic pregnancy, appendicitis, dissecting aortic aneurysm, cerebral trauma, diabetic acidosis and inflammation of pancreas from a perforating peptic ulcer. Rarely, combination of amylase with an immunoglobulin produces elevated serum amylase activity (macro amylasemia) because the large molecular complex is not filtered by the glomerulus.

Decrease in amylase level is seen in acute and chronic hepatitis, pancreatic insufficiency, advanced cystic fibrosis, pancreatectomy and occasionally in toxemia of pregnancy.

End Of Report

Please visit www.agilusdiagnostics.com for related Test Information for this accession

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